

1. Upper lobes sign

WHAT YOU SEE

A green airport road sign reading UPPER LOBES at the top-left — it labels the upper tier of the tarmac as the upper-lobe zone, anchoring the upper-lobe causes (CF/ABPA/TB).

WHAT IT ENCODES

Lobar distribution is a high-yield board discriminator for the underlying cause. UPPER-lobe-predominant bronchiectasis points to cystic fibrosis first, and also to ABPA and post-primary TB or sarcoid. The 'vicious cycle' mechanism is shared: impaired clearance → infection → neutrophilic inflammation → airway-wall destruction and permanent dilation. Mnemonic: think 'A' diseases up top — Aspergillus (ABPA), Acid-fast (TB), and CF.

BOARD TRAP

Don't reflexively assign ALL bronchiectasis to the upper lobes — idiopathic and post-infectious disease favor the LOWER lobes, so distribution actively narrows the differential rather than confirming a single cause.

2. Parker / CF planes

WHAT YOU SEE

A green PARKER LOBES sign standing over two upper-tier planes slashed with big red X's (one tagged SWEAT CHLORIDE) — the crossed-out planes and sweat-chloride label flag cystic fibrosis as the upper-lobe culprit.

WHAT IT ENCODES

Cystic fibrosis (CFTR mutation, autosomal recessive, ΔF508 most common) is the classic cause of UPPER-lobe bronchiectasis. Suspect it in a young patient with chronic sinopulmonary infection (Pseudomonas/Staph), pancreatic insufficiency with malabsorption, and male infertility from congenital absence of the vas deferens. Confirm with a sweat chloride >60 mmol/L and CFTR genetics.

BOARD TRAP

Don't default to CF in an OLDER adult with LOWER-lobe disease — that picture is post-infectious or idiopathic; conversely, missing CF in a young adult with upper-lobe disease and infertility/sinus disease is the under-diagnosis trap (mild CFTR variants can present in adulthood).

3. MAC plane (Lady Windermere)

WHAT YOU SEE

A green twin-prop plane in mid-field stenciled MAC, marked with radiation/biohazard symbols and a no-smoking sign, an elderly thin woman boarding — the demure old lady plus the nontuberculous-mycobacteria biohazard marks = MAC 'Lady Windermere' nodular bronchiectasis.

WHAT IT ENCODES

Mycobacterium avium complex (a nontuberculous mycobacterium) causes NODULAR bronchiectasis of the right middle lobe and lingula — the classic 'Lady Windermere syndrome' in older, thin, nonsmoking women who habitually suppress their cough. Diagnose with HRCT (tree-in-bud + cylindrical bronchiectasis) plus repeated sputum AFB cultures; treat with a macrolide-based multidrug regimen for ~12 months after culture conversion.

BOARD TRAP

Nodular RML/lingular bronchiectasis in an elderly woman is MAC, not simple post-pneumonic scarring — and the answer is multidrug NTM therapy, not a single macrolide (monotherapy breeds macrolide resistance).

4. MidLung sign

WHAT YOU SEE

A green MIDLUNG road sign on the left tarmac — it tags the middle-lobe/lingula zone, the signature distribution of MAC nodular disease.

WHAT IT ENCODES

Middle-lobe and lingular involvement is the signature distribution of MAC nodular bronchiectasis, reinforcing the Lady Windermere pattern. The right middle lobe is also anatomically prone to recurrent collapse/infection because of its narrow long bronchus ('right middle lobe syndrome').

BOARD TRAP

Isolated middle-lobe/lingular disease in a thin elderly woman should prompt NTM sputum cultures rather than being dismissed as recurrent ordinary pneumonia.

5. ABPA control tower

WHAT YOU SEE

The central airport control tower with a wooden double door labelled ABPA — the tower sits dead-center (central/perihilar location) just as ABPA causes central bronchiectasis.

WHAT IT ENCODES

Allergic bronchopulmonary aspergillosis is a hypersensitivity reaction to Aspergillus colonizing the airways, occurring in patients with ASTHMA or CF. It produces CENTRAL/perihilar bronchiectasis with mucoid impaction. Diagnostic clues: total IgE >1000 IU/mL, positive Aspergillus-specific IgE and IgG, peripheral eosinophilia, and fleeting infiltrates.

BOARD TRAP

PERIPHERAL bronchiectasis argues AGAINST ABPA — ABPA is classically CENTRAL — so choosing ABPA for a peripheral-predominant pattern is wrong; the central location is the discriminator.

6. Aspergillus monster

WHAT YOU SEE

A green spiky fungal monster with tentacles perched atop the ABPA door — the literal Aspergillus mold colonizing the central airway, the hypersensitivity trigger of ABPA.

WHAT IT ENCODES

Aspergillus fumigatus hypersensitivity (Type I and Type III reactions) drives ABPA. Workup: very high total IgE (often >1000 IU/mL), elevated Aspergillus-specific IgE/IgG, and eosinophilia >500/μL. Treatment is systemic corticosteroids to suppress the immune response, with itraconazole/voriconazole as a steroid-sparing antifungal adjunct.

BOARD TRAP

ABPA is a HYPERSENSITIVITY reaction, not invasive aspergillosis or an aspergilloma — treat with steroids ± azole antifungal, NOT surgical resection or amphotericin; treating it as invasive infection is the categorization trap.

7. Immunodeficiency cabinets

WHAT YOU SEE

Tall stacked file cabinets/lockers beside the control tower, near a tag reading ALMOST ALWAYS REQUIRED — the bank of immunoglobulin 'files' represents humoral immunodeficiency (CVID) that must be screened for.

WHAT IT ENCODES

Humoral (antibody) immunodeficiency — common variable immunodeficiency or other hypogammaglobulinemia — causes recurrent encapsulated-organism sinopulmonary infections that scar airways into bronchiectasis. Screen with quantitative immunoglobulins (IgG, IgA, IgM) and vaccine-response titers; treat with immunoglobulin replacement.

BOARD TRAP

In recurrent infections plus bronchiectasis, don't stop at repeated antibiotic courses — failing to MEASURE IgG/IgA/IgM and missing CVID (a treatable, IVIG-responsive cause) is the classic omission.

8. Lower lobes sign

WHAT YOU SEE

A green LOWER LOBES sign over the right-side scene with its bonsai tree and basket-weave plane — it marks the lower-lobe zone where idiopathic/post-infectious/aspiration causes cluster.

WHAT IT ENCODES

LOWER-lobe-predominant bronchiectasis suggests idiopathic, post-infectious (childhood pneumonia/pertussis/measles), recurrent ASPIRATION, or immunodeficiency causes — gravity favors basal infection and aspiration. This is the opposite anchor to the upper-lobe (CF/ABPA/TB) group.

BOARD TRAP

Lower-lobe disease should steer AWAY from CF/ABPA and toward post-infectious, aspiration, or immunodeficiency etiologies — anchoring on CF for basal disease is the distribution trap.

9. Wheelchair / chronic patient

WHAT YOU SEE

An elderly patient hunched in a wheelchair on the right tarmac — the chronically disabled, recurrently infected patient whose same-lobe pneumonias should prompt a hunt for a fixed structural/obstructive cause.

WHAT IT ENCODES

Depicts the chronically disabled, recurrently infected patient. A key board pearl: recurrent pneumonia in the SAME lobe should trigger a search for an underlying structural cause — localized bronchiectasis, an endobronchial obstructing lesion (tumor/foreign body), or extrinsic compression.

BOARD TRAP

Recurrent pneumonia confined to one lobe is NOT just bad luck — overlooking a fixed obstruction (especially endobronchial tumor in a smoker) and not obtaining bronchoscopy/CT is the workup trap.

10. HIV sign

WHAT YOU SEE

A red road sign reading HIV at the bottom-right — the red sign flags HIV/immunosuppression as a driver of recurrent infections leading to bronchiectasis.

WHAT IT ENCODES

HIV and other cell-mediated/combined immunodeficiency states predispose to recurrent bacterial and opportunistic infections (including NTM and recurrent pneumonia) that lead to bronchiectasis. Bronchiectasis can also reflect chronic immune dysregulation in poorly controlled HIV.

BOARD TRAP

Don't forget to consider HIV/immunosuppression as a driver of bronchiectasis — attributing recurrent infections to 'COPD' alone without checking immune status can miss a treatable underlying cause.

11. Kartagener's sign

WHAT YOU SEE

A road sign reading KARTAGENER'S slashed with a red X at the bottom-right — names primary ciliary dyskinesia (the triad of bronchiectasis + sinusitis + situs inversus).

WHAT IT ENCODES

Primary ciliary dyskinesia (autosomal recessive dynein-arm defect) impairs mucociliary clearance. Kartagener triad = bronchiectasis + chronic sinusitis + situs inversus (dextrocardia), plus infertility from immotile sperm/dysfunctional fallopian cilia. Confirm with low nasal nitric oxide screening and ciliary ultrastructure on biopsy.

BOARD TRAP

Situs inversus + chronic sinopulmonary infection = Kartagener/PCD, NOT cystic fibrosis — confirm with nasal NO and ciliary biopsy; defaulting to CF (or missing the dextrocardia clue on CXR) is the trap.

12. 25-50% idiopathic banner

WHAT YOU SEE

A large banner stretched across the bottom fence reading 25-50% IDIOPATHIC — the headline statistic that a quarter to half of cases never find a cause.

WHAT IT ENCODES

Even after a complete etiologic workup, roughly 25–50% of bronchiectasis remains IDIOPATHIC. Among identifiable causes, POST-INFECTIOUS is the most common. Across all cases, chronic airway colonization with *Pseudomonas aeruginosa* (and mucoid strains) marks worse prognosis and more frequent exacerbations.

BOARD TRAP

A negative cause workup does NOT mean you missed something — idiopathic bronchiectasis is genuinely common, so endlessly re-testing rather than managing airway clearance/colonization is the trap.